

PAPER MEDICAL CENTER · CARDIOLOGY DEPARTMENT

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Anamnesis & Initial Assessment Report

Document ref: ANAM-2026-04812 · Encounter: 2026-05-20 09:14 · 7 of 9 pages

PATIENT DEMOGRAPHICS

PATIENT NAME	MRN	DATE OF BIRTH & AGE	INSURANCE	REFERRING CLINICIAN
Reyes, Marcus J.	MR-04812-26	1972-08-14 · 53 y	BlueCare Premier	Dr. A. Patel, GP
Preferred: "Marc"	Paper Health ID	Sex: Male	Policy #BC-77821-3	Bayview Family Clinic

BLOOD PRESSURE

138 / 88
· stage 1 hypertension

HEART RATE

82 bpm
within normal range

BMI

28.7
overweight

SPO₂

97%
normal

TEMPERATURE

36.8°C
afebrile

CHIEF COMPLAINT & HISTORY OF PRESENT ILLNESS

Chief complaint: Episodic central chest pressure with mild shortness of breath, worse on exertion. Onset approximately 6 weeks ago, increasing in frequency over the past 10 days.

Patient describes a substernal pressure-like sensation, non-radiating, lasting 5–10 minutes, reproducibly triggered by climbing two flights of stairs or carrying groceries. Pain resolves with rest within 10 minutes. No associated diaphoresis, nausea, or palpitations. No syncope. Sleep is undisturbed. No recent infections or fevers.

RISK FACTOR SCREENING (MODIFIED WHO ROSE QUESTIONNAIRE)

Q1 · CORONARY SYMPTOMS

Have you ever had pain or discomfort in your chest?

YES Patient confirms pressure-like episodes on exertion, see HPI above.

Q5 · SMOKING

Do you currently smoke tobacco?

NO Quit 8 years ago. Prior 15 pack-year history (15 cigs/day × 22 years).

Q2 · SEVERITY

Does it ever make you stop walking?

YES "I have to slow down and lean on something for a minute."

Q6 · DIABETES

Have you been told you have diabetes?

PRE-DIABETIC HbA1c 6.1% in 2025. Not on metformin.

Q3 · RELIEF

Does the pain go away when you stand still?

YES Resolves within 5–10 minutes of rest.

Q7 · HYPERTENSION

Are you being treated for high blood pressure?

YES Lisinopril 10 mg daily since 2023.

Q4 · LOCATION

Where do you feel the discomfort?

Substernal · non-radiating · no jaw / arm involvement.

Q8 · FAMILY HISTORY

First-degree relative with early cardiac disease?

YES Father MI at age 54. Brother CABG at age 58.

COMPOSITE RISK PROFILE

Family history CAD

Hypertension

Pre-diabe

Former
smoker

BMI
28.7

No
CKD

No
prior
MI

No
arrhythmia

Active
lifestyle
(3x/wk
walk)

No
EtOH
abuse

Framingham 10-year CVD risk estimate: 17.4% (high). ASCVD pooled cohort risk: 15.8%. Recommend statin initiation per ACC/AHA 2018 guidelines.

CURRENT MEDICATIONS & ALLERGIES

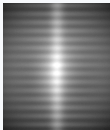
Active medications

Drug	Dose	Freq	Since
Lisinopril	10 mg	QD	2023-02
Atorvastatin	20 mg	QHS	2024-09
Aspirin (low-dose)	81 mg	QD	2024-12
Vitamin D.	1000 IU	QD	2022-01

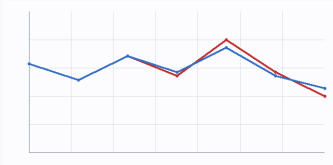
Allergies & intolerances

Agent	Reaction	Severity
Penicillin	Urticaria	Severe
Shellfish	Lip swelling	Mild
Latex	Contact rash	Mild

IMAGING & DIAGNOSTIC STUDIES

PA CHEST X-RAY 12/25/2026-05-20 09:38 Cardiothoracic ratio 0.51 — borderline  Mild cardiomegaly, no consolidation, lung fields clear. No pleural effusion. Mediastinum not widened.	12-LEAD ECG 12/25/2026-05-20 09:38 NSR 78 bpm · borderline LVH  Normal sinus rhythm. Mild voltage criteria for left ventricular hypertrophy. No ST elevation, no Q waves.
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HOME VITALS TREND 12/25/2026-05-20 09:38 LAST 7 DAYS

SOURCE: PATIENT-RECORDED 12/25/2026-05-20 09:38 · VALIDATED CUFF (OMRON HEM-7156T)  . Systolic BP (mmHg, scale 110–145) . Heart rate (bpm, scale 60–95)

LABORATORY RESULTS (DRAWN 2026-05-20)

Analyte	Result	Unit	Reference range	Flag
Total cholesterol	224	mg/dL	< 200	HIGH
LDL cholesterol	148	mg/dL	< 100	HIGH
HDL cholesterol	38	mg/dL	> 40	LOW
Triglycerides	192	mg/dL	< 150	HIGH
HbA1c	6.1	%	< 5.7	HIGH
Fasting glucose	108	mg/dL	70–99	HIGH
Creatinine	0.94	mg/dL	0.7–1.3	OK
eGFR	92	mL/min	> 60	OK
hs-Troponin I	8	ng/L	< 19	OK
BNP	42	pg/mL	< 100	OK

ASSESSMENT & IMPRESSION

Primary: Probable stable angina pectoris (exertional class II). Multifactorial cardiovascular risk profile dominated by family history, dyslipidemia, and uncontrolled hypertension.

Secondary considerations: Pre-diabetes mellitus (HbA1c 6.1%); overweight (BMI 28.7); borderline LVH on ECG; mild cardiomegaly on CXR — likely chronic hypertensive remodelling.

Reassuring findings: Negative high-sensitivity troponin and BNP — acute coronary syndrome not currently active. Renal function preserved.

RECOMMENDED PLAN

- 1. **Confirm ischemia.** Schedule exercise stress test with myocardial perfusion imaging within 14 days. If positive or equivocal, proceed to coronary CT angiography.
- 2. **Optimize antihypertensive therapy.** Increase lisinopril to 20 mg QD; recheck BP in 4 weeks. Add amlodipine 5 mg if SBP > 135 at follow-up.
- 3. **Intensify lipid therapy.** Atorvastatin 40 mg QHS. Repeat full lipid panel in 8 weeks. Discuss PCSK9 inhibitor if LDL > 100 at recheck.
- 4. **Lifestyle counselling.** Mediterranean diet referral to nutrition. Goal: 5% body weight loss over 6 months. Continue 30-min walks 5x/week.
- 5. **Pre-diabetes management.** Recheck HbA1c in 3 months. Refer to diabetes prevention program.
- 6. **Patient safety net.** Provide chest-pain action card with symptom thresholds for ER presentation. Sublingual nitroglycerin 0.4 mg SL PRN, max x3 with 5-min interval.
- 7. **Follow-up cardiology visit.** In 6 weeks (post stress test & lipid recheck).

Clinician statement

I have personally reviewed the history, examined the patient, and discussed the assessment and recommended plan. Patient demonstrated understanding (teach-back verified) and consented to the proposed investigations.

Report prepared automatically from the Paper Anamnesis questionnaire and finalised by the attending clinician at 2026-05-20 11:02 PT. Document checksum: 6f9e83...b4c1.

ATTENDING PHYSICIAN
Dr. Helena Costa, MD Board-certified
Cardiologist NPI 1235647890 License
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